IMPLEMENTATION OF A POLICY COMMITTEE AT THE PHOENIX INDIAN MEDICAL CENTER

Prepared by: Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™

Policy and Procedure Nurse Consultant, COR II, Phoenix Indian Medical Center

Date: April 7, 2025

I. Table of Contents

I. TABLE OF CONTENTS 1

II. EXECUTIVE MEMORANDUM 2

III. FORMAL TIMELINE AND LEGAL OPINION: FAILURE OF PHOENIX INDIAN MEDICAL CENTER TO ESTABLISH A POLICY COMMITTEE 4

IV. I. TIMELINE OF POLICY COMMITTEE REQUESTS AT PHOENIX INDIAN MEDICAL CENTER (PIMC) 4

V. II. LEGAL OPINION ON CONTINUED NONCOMPLIANCE AND BEST PRACTICE STANDARDS 5

A. Legal and Regulatory Standards 5

VI. REFERENCES 7

VII. EXPANDED SECTION: REGULATORY AND PROFESSIONAL STANDARDS SUPPORTING A POLICY COMMITTEE 9

1. Centers for Medicare & Medicaid Services (CMS) 9
2. The Joint Commission (TJC) 9
3. Clinical Laboratory Improvement Amendments (CLIA) 9
4. College of American Pathologists (CAP) 9
5. American Medical Association (AMA) 9
6. American Nurses Association (ANA) 9

VIII. SUPPORTING WHITE PAPERS AND GUIDANCE DOCUMENTS 10

IX. COMPREHENSIVE LIST OF SUPPORTING WHITE PAPERS AND GUIDANCE DOCUMENTS 11

X. EXECUTIVE UPDATE: POLICY COMMITTEE ALIGNMENT WITH IHS STRATEGIC PLAN (FY 2025–2029) 12

Strategic Goal 1: Be a Leading Health Care Organization 12
Strategic Goal 2: Ensure Comprehensive, Culturally Respectful Health Care Services 12
Strategic Goal 3: Optimize Operations Through Effective Stewardship 12
Strategic Goal 4: Promote Proactive Intergovernmental and External Relationships 12

XI. FORMAL BRIEFING MEMO: IMPACT OF HQ ACCESS RESTRICTIONS ON POLICY GOVERNANCE 13

Overview: 13
Risk Summary: 13
Recommendation: 13
Immediate Action Items: 14

II. Executive Memorandum

To: PIMC Executive Leadership, IHS Area Office, and HHS Oversight Authorities

From: Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™

Policy and Procedure Nurse Consultant, COR II

Subject: Immediate Implementation of Evidence-Based Policy Committee at PIMC

Date: April 7, 2025

This memorandum formally documents and reinforces the critical requirement for Phoenix Indian Medical Center (PIMC) to establish a formal, multidisciplinary Policy Committee without further delay. The absence of such a structure directly violates federal regulatory mandates, accreditation standards, and the Indian Health Service (IHS) Strategic Plan for FY 2025–2029.

Continued reliance on departmental 'owners' to manage, edit, and approve their own policies has been rendered obsolete, particularly in light of the recent HQ directive limiting system policy access to 5–10 users. Without a centralized, evidence-based governance body, PIMC remains in violation of the following:

- CMS Conditions of Participation (42 CFR § 482.21)
- Joint Commission Standards (LD.04.01.05; RC.01.01.01)
- HIPAA, CLIA, and CAP documentation regulations
- Indian Health Manual (IHM), and the HHS Records Management Policy

Furthermore, avoidance of this best practice exposes PIMC, IHS, HHS, and the federal government to legal risk under the False Claims Act and increases the likelihood of sentinel events and compliance findings.

The IHS Strategic Plan explicitly mandates high-reliability operations, evidence-based practice, standardization, and interdepartmental accountability—none of which can be achieved without a Policy Committee.

Effective immediately, I formally recommend that leadership take the following corrective actions:

1. Establish and charter a multidisciplinary Policy Committee with full governance authority.

2. Assign a Policy System Administrator to manage technical system access under the new HQ limitations.

3. Develop and enforce a standardized Policy Review and Approval SOP.

4. Submit an exception request or risk memo to HQ outlining the operational hazards of limited user access without a policy committee.

Respectfully submitted,

Digitally signed by Kevin D. Turnage -S
Date: 2025.04.07 13:25:03 -07'00'

Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™

Policy and Procedure Nurse Consultant, COR II

Phoenix Indian Medical Center

III. Formal Timeline and Legal Opinion: Failure of Phoenix Indian Medical Center to Establish a Policy Committee

IV. Timeline of Policy Committee Requests at Phoenix Indian Medical Center (PIMC)

Date	Action / Communication	Summary
Nov 29, 2023	Initial formal submission	Submitted documents including a draft Hospital Policy, Policy Committee Charter, Bylaws, and legal citations.
Dec 17, 2023	Follow-up submission	Resubmitted Policy Committee proposal materials with emphasis on compliance and risk mitigation.
Feb 22, 2024	Compliance reminder	Reiterated federal and accreditation requirements for regular policy review.
May 21, 2024	Updated draft and escalation	Submitted revised Policy Management Policy draft; emphasized legal exposure due to inaction.
Jun 14, 2024	Resubmission	Renewed call for committee formation due to continued inaction and compliance issues.
Jun 24, 2024	Documentation update	Provided updated recommendations citing CMS and Joint Commission standards.
Sep 17, 2024	Communication with CQO (Clinton Pope)	Highlighted deficiencies in the Policy Management draft, particularly lack of EBM and stakeholder involvement.
Jan 16–17, 2025	Joint communication with Dr. Endreya McCabe	Provided comprehensive analysis of policy compliance gaps and the necessity of a policy committee; Dr. McCabe acknowledged support.

Feb 14, 2025	Formal re-submission	Provided previously submitted documents again for review and action.
Mar 10, 2025	Compliance escalation	Urged executive action to adopt committee infrastructure due to continued inaction and risk exposure.
Mar 18, 2025	Immediate call to action	Requested Executive Committee presentation and written documentation of decision-making rationale.
Mar 21, 2025	Final documented justification	Delivered a legal and accreditation analysis showing why the current policy model violates federal law and standards; reiterated the need for a Policy Committee.

V. Legal Opinion on Continued Noncompliance and Best Practice Standards

A. Legal and Regulatory Standards

1. CMS Conditions of Participation (42 CFR § 482.21):
Hospitals must maintain active, reviewed, and up-to-date policies. Lack of oversight can lead to termination from Medicare and financial penalties.

2. The Joint Commission (TJC) Standards:
Accreditation requires a multidisciplinary policy governance structure. Noncompliance may result in accreditation loss, which affects funding and reputation.

3. HIPAA and EMTALA:
These laws mandate timely policy updates to ensure legal compliance in patient privacy and emergency care.

4. False Claims Act (31 U.S.C. § 3729 et seq.):
Using outdated policies can result in fraudulent billing or substandard care, exposing PIMC to civil liability and investigation.

5. Federal Records Act and NARA Guidelines:
Require lifecycle tracking of documents, which demands a structured policy review process.

B. Legal Assessment of Agency Inaction

Despite formal submissions spanning 16 months, leadership has failed to act. This constitutes negligence and may result in:
- Civil and regulatory liability.
- Enforcement actions from HHS, CMS, and the OIG.
- Whistleblower protections for employees and OSC intervention.

C. National Best Practices

Leading institutions including the Mayo Clinic, Cleveland Clinic, VA, and DoD systems maintain formal policy committees to ensure:
- Evidence-based compliance,
- Legal, operational, and clinical review, and
- Documentation of all policy changes.

Conclusion:

PIMC's refusal to establish a Policy Committee directly contravenes federal standards and exposes patients to preventable risk. Immediate corrective action is warranted to avoid legal, financial, and reputational harm.

VI. References

Centers for Medicare & Medicaid Services. (2022). *Conditions of participation for hospitals*. U.S. Department of Health & Human Services. https://www.cms.gov/Regulations-and-Guidance

Cleveland Clinic. (2021). *Policy management system guidelines*. Cleveland Clinic. https://my.clevelandclinic.org

Indian Health Service. (n.d.). *Indian Health Manual, Part 1, Chapter 1; Part 5, Chapter 15*. U.S. Department of Health & Human Services. https://www.ihs.gov/ihm/

Joint Commission. (2021). *Hospital accreditation standards manual*. Oakbrook Terrace, IL: The Joint Commission.

Mayo Clinic. (2020). *Policy review and approval process*. Mayo Clinic. https://www.mayoclinic.org

McGowan, B., Friedman, E., & Hill, K. (2020). Healthcare policy management and compliance: Ensuring patient safety and legal adherence. *Journal of Healthcare Administration, 33*(4), 112–129. https://doi.org/10.1016/j.jha.2020.04.007

Pelayo, L., André, J. B., & Légeron, P. (2022). Hospital policy implementation and staff training effectiveness. *Journal of Hospital Administration, 41*(2), 85–102. https://doi.org/10.1016/j.jha.2022.02.004

Veterans Health Administration. (2022). *Evidence-based policy subcommittee drives use of high-impact evidence*. QUERI News. https://www.queri.research.va.gov

Agency for Healthcare Research and Quality. (2008). *Patient safety and quality: An evidence-based handbook for nurses* (Vol. 1). U.S. Department of Health and Human Services. https://www.ahrq.gov/patient-safety/resources/index.html

Australian Commission on Safety and Quality in Health Care. (2017). *National model clinical governance framework*. https://www.safetyandquality.gov.au/publications-and-resources

Canadian Patient Safety Institute. (2016). *Patient safety and quality improvement guide*. https://www.patientsafetyinstitute.ca

Dainty, K. N., Scales, D. C., Sinuff, T., & Zwarenstein, M. (2017). A qualitative study of hospital policy development processes in Ontario, Canada. *BMJ Open, 7*(1), e013649. https://doi.org/10.1136/bmjopen-2016-013649

Gagliardi, A. R., Brouwers, M. C., Palda, V. A., Lemieux-Charles, L., & Grimshaw, J. M. (2011). How can we improve guideline use? A conceptual framework of implementability. *Implementation Science, 6*, 26. https://doi.org/10.1186/1748-5908-6-26

Institute for Healthcare Improvement. (2012). *Leadership guide to patient safety*. https://www.ihi.org/resources/Pages/IHIWhitePapers/Achieving-Health-Equity.aspx

Leotsakos, A., Zheng, H., Croteau, R., Loeb, J. M., & Sherman, H. (2014). Standardization in patient safety: The WHO High 5s project. *International Journal for Quality in Health Care, 26*(2), 109–116. https://doi.org/10.1093/intqhc/mzu010

National Health Service England. (2020). *NHS policy governance handbook*. NHS Improvement.

Pronovost, P. J., Berenholtz, S. M., & Needham, D. M. (2008). Translating evidence into practice: A model for large-scale knowledge translation. *BMJ, 337*, a1714. https://doi.org/10.1136/bmj.a1714

The Joint Commission. (2024). *Hospital accreditation standards*. The Joint Commission.

World Health Organization. (2014). *Patient safety: A World Alliance for Safer Health Care—High 5s project interim report*. https://apps.who.int/iris/handle/10665/128810

Zingg, W., Castro-Sánchez, E., Secci, F. V., Edwards, R., Drumright, L., Sevdalis, N., & Holmes, A. (2016). Innovative tools for quality assessment: Integrated review of antimicrobial stewardship interventions. *Clinical Microbiology and Infection, 22*(8), 694–700. https://doi.org/10.1016/j.cmi.2016.04.009

VII. Expanded Section: Regulatory and Professional Standards Supporting a Policy Committee

1. Centers for Medicare & Medicaid Services (CMS)

Conditions of Participation for Hospitals (42 CFR § 482) require hospitals to maintain a governance structure that ensures systematic policy review and quality assurance activities (Centers for Medicare & Medicaid Services, 2022). The CMS State Operations Manual (Appendix A) emphasizes the necessity of regularly reviewed, dated, and approved policies. CMS Quality, Safety & Oversight memoranda further underscore the role of organizational policy governance in avoiding sentinel events and ensuring safe outcomes.

2. The Joint Commission (TJC)

The Comprehensive Accreditation Manual for Hospitals (CAMH, 2024 Edition) includes standards such as LD.04.01.05 and RC.01.01.01, which require leadership to ensure that hospital policies are implemented and regularly reviewed. Sentinel Event Alerts like SEA #57 highlight the leadership's role in policy oversight to prevent harm and ensure accountability (The Joint Commission, 2019).

3. Clinical Laboratory Improvement Amendments (CLIA)

CLIA regulations (42 CFR § 493 Subpart K) mandate that clinical laboratory policies be approved and reviewed biennially. Failure to comply may lead to suspension of certifications, affecting diagnostic and treatment capabilities (CMS CLIA, 2022).

4. College of American Pathologists (CAP)

CAP Accreditation Checklists require labs to maintain written and reviewed procedures across all testing phases. Noncompliance is a common source of deficiency citations, reinforcing the importance of formal policy review systems.

5. American Medical Association (AMA)

The AMA Code of Medical Ethics Opinion 1.1.6 calls for physicians to engage in institutional efforts to review clinical care policies. Additionally, AMA Organizational Principles promote physician involvement in policy development and structured governance.

6. American Nurses Association (ANA)

The ANA Code of Ethics (2015) underscores nurses' responsibilities to participate in policy development (Provision 7) and to maintain professional integrity through structured policy systems (Provision 9). The ANA Position Statement (2016) stresses the obligation to report unsafe policies and highlights the need for formal governance structures.

VIII. Supporting White Papers and Guidance Documents

Key white papers and reports from agencies such as AHRQ, WHO, IHI, and ANA support centralized policy governance. These documents advocate for standardized, evidence-based policy development as a core function of healthcare safety and quality improvement.

IX. Comprehensive List of Supporting White Papers and Guidance Documents

Agency for Healthcare Research and Quality (AHRQ) (2008). Patient Safety and Quality: An Evidence-Based Handbook for Nurses. Highlights the role of formal organizational structures—including policy review—in ensuring patient safety. Link: https://www.ahrq.gov/patient-safety/resources/index.html

Institute for Healthcare Improvement (IHI) (2012). Leadership Guide to Patient Safety. Emphasizes leadership responsibility in creating and maintaining governance structures such as policy committees to ensure quality and prevent harm. Link: https://www.ihi.org/resources/Pages/IHIWhitePapers/LeadershipGuidetoPatientSafety.aspx

World Health Organization (WHO) (2014). High 5s Project Interim Report: Standardization in Patient Safety. Discusses standardization through structured procedures and oversight—key responsibilities of policy committees. Link: https://apps.who.int/iris/handle/10665/128810

American Nurses Association (ANA) (2022). Shared Governance Handbook. Advocates for nurse participation in governance, including through policy and procedure committees. Link: https://www.nursingworld.org

Canadian Patient Safety Institute (CPSI) (2016). Patient Safety and Quality Improvement Guide. Provides strategic policy development and review recommendations, encouraging formal committee oversight. Link: https://www.patientsafetyinstitute.ca

Australian Commission on Safety and Quality in Health Care (2017). National Model Clinical Governance Framework. Offers a model for clinical governance that includes policy committee functions for accountability and performance. Link: https://www.safetyandquality.gov.au

The Joint Commission (2017). Sentinel Event Alert #57: The Essential Role of Leadership in Developing a Safety Culture. Urges leadership to form structures (like policy committees) to oversee safe, compliant operations. Link: https://www.jointcommission.org

X. Executive Update: Policy Committee Alignment with IHS Strategic Plan (FY 2025–2029)

The establishment of a formal Policy Committee at Phoenix Indian Medical Center (PIMC) directly advances the IHS Strategic Plan for Fiscal Years 2025–2029. A Policy Committee operationalizes core aspects of quality, safety, accountability, and equity. Below is a concise alignment summary between strategic goals and the committee's functions.

Strategic Goal 1: Be a Leading Health Care Organization

- Implements workplace learning through regulatory-informed policy updates.
- Drives system safety by standardizing and disseminating safety-related procedures.
- Supports standardized outcomes via uniform policy metrics and review cycles.

Strategic Goal 2: Ensure Comprehensive, Culturally Respectful Health Care Services

- Enhances onboarding, staff clarity, and retention through standardized role policies.
- Fosters shared governance, boosting employee engagement and mission connection.
- Removes access barriers through telehealth, scope of practice, and culturally adapted procedures.

Strategic Goal 3: Optimize Operations Through Effective Stewardship

- Improves transparency and accountability via centralized policy oversight.
- Ensures operational and infrastructure readiness is supported by current SOPs.
- Drives evidence-based decision-making and supports IHS goals of High Reliability.

Strategic Goal 4: Promote Proactive Intergovernmental and External Relationships

- Facilitates integration of Tribal and UIO feedback into PIMC policies.
- Respects Tribal self-determination by supporting community-partnered policy design.

XI. Formal Briefing Memo: Impact of HQ Access Restrictions on Policy Governance

To: Executive Leadership, Compliance Office, and Clinical Directors

From: Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™

Subject: Urgent Briefing on HQ Directive to Limit Policy System Access and Discontinue Owner-Managed Updates

Date: April 7, 2025

Overview:

Headquarters has issued a directive to reduce system policy editing access to only 5–10 accounts across the facility. Additionally, departmental policy owners will no longer have the ability to edit or manage their own policies directly within the system. This structural change fundamentally alters how policies are maintained, reviewed, and updated at Phoenix Indian Medical Center (PIMC).

Risk Summary:

1. Timeliness Risk – Centralizing access without a defined governance model delays necessary updates and endangers compliance.

2. Regulatory Risk – Noncompliance with CMS, TJC, CLIA, and CAP standards could result in penalties, loss of accreditation, or funding disruption.

3. Accountability Gap – Removing owner autonomy without a replacement structure creates a void in oversight and version control.

4. Representation Deficit – Restricting editing to 5–10 users may exclude critical stakeholders including clinical, nursing, and legal experts.

Recommendation:

To mitigate these significant risks, it is imperative that PIMC immediately establish a formal, multidisciplinary Policy Committee. This committee will act as the centralized governance authority for all hospital policies, responsible for:

- Reviewing and approving all new or revised policies

- Managing access control and policy workflows in the limited-user environment

- Ensuring adherence to evidence-based practice, federal law, and accreditation standards

- Coordinating with HQ on any technical or procedural exceptions

This structure ensures that PIMC maintains operational, legal, and clinical continuity under the new access constraints, and supports full alignment with national best practices.

Immediate Action Items:

1. Establish the Policy Committee with representatives from Nursing, Medicine, Compliance, Risk, Legal, and Quality.

2. Appoint a Policy System Administrator accountable to this committee.

3. Develop a Policy Workflow SOP tailored to HQ's limitations.

4. Submit a formal exception request to HQ if governance needs exceed current technical permissions.

Respectfully submitted,

Hon. Kevin D. Turnage, MSN Ed., RN, AMB-BC™, NE-BC®, NHDP-BC™, NPD-BC™

Policy and Procedure Nurse Consultant, COR II